

The IBD Partner Guide

What the people closest to you actually need to know to help, not hover

This one is meant to be handed over, to a partner, parent, close friend or housemate, rather than kept. Explaining Crohn's or colitis from scratch every time it comes up is exhausting; this page does the first pass so the conversation can start further along.

THE BASICS

What Crohn's and Colitis Actually Are

- Long-term conditions where the digestive tract becomes inflamed, not caused by stress, diet or anything the person did
- Not the same as IBS: IBD involves real, visible inflammation and damage, and is diagnosed by scans, scopes and blood or stool tests
- Symptoms come and go in flares and remission; a good week doesn't mean it's gone, and a bad week doesn't mean something was done wrong
- There is no cure yet, but treatment can bring long periods of feeling well

BE USEFUL, NOT ANXIOUS

What Actually Helps

- Flexibility with plans, and no visible frustration when they change at short notice
- Knowing where the nearest toilet is when you're out together, without being asked
- Offering specific practical help ("I'll do the food shop") rather than a general "let me know if you need anything"
- Taking fatigue seriously even when nothing looks obviously wrong; it is a real, common part of IBD, not tiredness that a good night's sleep fixes

WELL-MEANT, OFTEN UNHELPFUL

What Tends Not to Help

What often gets said	Why it lands badly
"You don't look sick."	IBD is often invisible from the outside; this can read as doubt.
Unsolicited diet advice	There is no single "IBD diet," and most people have already tried the obvious suggestions.
"Have you tried just relaxing more?"	Stress can affect symptoms, but it does not cause the disease, and the comment can feel dismissive.
Pushing for symptom detail in public	Bowel symptoms are personal; let them choose what and when to share.

WHAT A FLARE CAN LOOK LIKE

On a Bad Day

A flare isn't always dramatic. It can look like cancelling plans at the last minute, needing to be near a toilet, eating very little, or being noticeably more tired and short-tempered than usual. None of that is about you.

Practical ways to help on a bad day

Offer to drive rather than suggest public transport; run an errand without being asked; keep plans low-key and easy to leave early; and don't take a cancelled evening personally.

ASKING WELL

Talking About It

- Ask how they're doing today, not just how "the IBD" is; the two aren't always the same
- It's fine to ask questions; most people would rather explain than be guessed about
- It's also fine for them to say "not right now," especially in front of others
- If they mention a new symptom or a change, encourage them to log it and mention it to their team rather than diagnosing it yourselves

THE CLOSER RELATIONSHIP

For Partners Specifically

Fatigue, pain and medication timing can all affect intimacy and shared plans, and that is a normal part of living with a long-term condition together, not a sign anything is wrong between you. Being open about energy levels and good days versus bad days tends to work better than guessing, and a flare is a reason for patience, not a personal rejection.

FOR BOTH OF YOU

More Support From VanceHealthHub

- **The IBD Flare Survival Guide:** what actually helps during the first 48 hours of a flare
- **IBD Fatigue: 15 Things That Can Help:** why fatigue is real, and what tends to help
- **Health Record:** where they can log symptoms and patterns in one private place
- **VANCE-Ai:** a starting point for questions between appointments, not a substitute for their care team

None of these replace their GP, gastroenterologist or dietitian, and nothing on VanceHealthHub should be used to diagnose or treat a condition.

THE EVIDENCE BEHIND IT

References

Crohn's & Colitis UK · Guts UK

1. Crohn's & Colitis UK. For Family and Friends.
2. Czuber-Dochan W, et al. Review article: describing and quantifying the impact of fatigue on people living with inflammatory bowel disease, a European patients' perspective. *Aliment Pharmacol Ther*, 2020;52(1):5-13.
3. Bernstein CN. Psychological Stress and Depression: Risk Factors for IBD? *Dig Dis*, 2016;34(1-2):58-63.

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